

DAPT for Non-Cardioembolic Ischemic Stroke

Acute Dual Antiplatelet Therapy (DAPT) Decision Flowchart and Regimen Timelines

TRIAL	TARGET POPULATION	LOADING DOSE (DAY 1)	DAPT DURATION	POST-DAPT
POINT Trial	NIHSS ≤ 3 or ABCD ² ≥ 4 . Within 12 hours of onset.	Clopidogrel 600 mg + Aspirin 162–325 mg	Clopidogrel 75mg qD + Aspirin 50–325mg qD for 90 days (as randomized)	Aspirin 81mg
CHANCE / INSPIRES	CHANCE: NIHSS ≤ 3 , ABCD ² ≥ 4 within 24h. INSPIRES: NIHSS ≤ 5 , ABCD ² ≥ 4 , $\geq 50\%$ stenosis within 72h.	Clopidogrel 300 mg + Aspirin 75–300 mg	Clopidogrel 75mg qD + Aspirin 75–100mg for 21 days	Clopidogrel 75mg (to Day 90)
CHANCE-2 Trial	CYP2C19 LOF carrier (*2/*3) + Minor stroke/TIA. Within 24h.	Ticagrelor 180 mg + Aspirin 75–300 mg	Ticagrelor 90mg BID + Aspirin 75–100mg for 21 days	Ticagrelor 90mg BID (to Day 90)
THALES Trial	NIHSS ≤ 5 or high-risk TIA (ABCD ² ≥ 6 or symptomatic stenosis) within 24h.	Ticagrelor 180 mg + Aspirin 300–325 mg	Ticagrelor 90mg BID + Aspirin 75–100mg for 30 days	Aspirin 81mg
SAMMPRIS Trial	Severe symptomatic atherosclerotic stenosis (70–99%) of a major intracranial artery.	Aspirin 325 mg + Clopidogrel 75 mg (no load)	Clopidogrel 75mg qD + Aspirin for 90 days	Aspirin

CYP2C19 Genotyping & Clopidogrel Resistance

- CYP2C19 LOF alleles reduce clopidogrel activation. When rapid genotype results are available, LOF status can guide ticagrelor-vs-clopidogrel selection; CHANCE-2 evidence applies to LOF carriers rather than mandating universal testing.

Safety

- **Duration — 21 vs 90 days:** Minor stroke (NIHSS ≤ 3) / high-risk TIA (ABCD² ≥ 4) → **21 days** ASA + clopidogrel (POINT/CHANCE), then single antiplatelet — benefit is concentrated in the first 21 days while bleeding risk rises beyond it. Severe symptomatic intracranial atherosclerotic stenosis (70–99%) → **90 days** ASA + clopidogrel (SAMMPRIS) plus intensive risk-factor control. THALES ASA + ticagrelor is a **30-day** regimen for NIHSS ≤ 5 / high-risk TIA. Short-course DAPT is endorsed by the 2021 secondary-prevention guideline and the 2026 AHA/ASA AIS guideline.
- **Post-Lytic / EVT Policy:** After IV thrombolysis (tenecteplase or alteplase), avoid antithrombotics for the first 24h until follow-up imaging excludes hemorrhage. EVT alone is not a blanket DAPT contraindication; stenting/angioplasty plans and hemorrhage risk drive the decision.

POINT: Johnston SC et al. *N Engl J Med.* 2018;379:215-225. [PMID: 29766750](#) | **CHANCE:** Wang Y et al. *N Engl J Med.* 2013;369:11-19. [PMID: 23803136](#)

CHANCE-2: Wang Y et al. *N Engl J Med.* 2021;385:2520-2530. [PMID: 34708996](#) | **INSPIRES:** Gao Y et al. *N Engl J Med.* 2023;389:2413-2424. [PMID: 38157499](#)

THALES: Johnston SC et al. *N Engl J Med.* 2020;383:207-217. [PMID: 32668111](#) | **SAMMPRIS:** Chimowitz MI et al. *N Engl J Med.* 2011;365:993-1003. [PMID: 21899409](#)